International Patient Summary (IPS)

One minimal clinical summary → coded with global standards → localised through a national or regional Core IG → understood wherever the patient goes.

A global standard for the essential health facts a clinician needs when a patient arrives from somewhere else. The IPS is **"a minimal and non-exhaustive set of basic clinical data of a patient, specialty-agnostic, condition-independent, but readily usable by all clinicians for unscheduled (cross-border) care."** Current editions: HL7 FHIR IPS 2.0 (October 2025) and ISO 27269:2025.

1 Key concepts

Minimal and universal

Problems, allergies and medicines first; usable by any clinician, in any specialty.

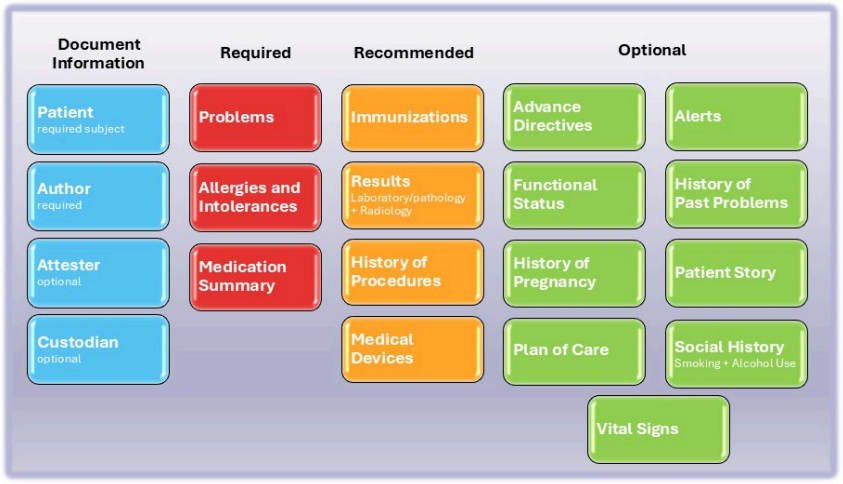
Built on global standards

One HL7 FHIR document: structured data plus narrative, coded in SNOMED CT and LOINC.

Localise, don't reinvent

Countries add a Core IG and local needs; a national summary stays a valid IPS.

2 The IPS at-a-glance



Read it left to right

Document information, then the **required** core, **recommended** and **optional** sections. Each carries readable narrative plus coded data.

Foundation

HL7 FHIR document (Bundle), SNOMED CT, LOINC and ISO 27269, stewarded by the Joint Initiative Council (CEN, HL7, IHE, ISO, SNOMED International).

Source: The IPS composition, HL7 FHIR International Patient Summary Implementation Guide.

3 Where it came from

2008–14	epSOS pilots cross-border patient summaries in Europe
2010	EU–US eHealth Memorandum of Understanding
2013–15	Trillium Bridge links EU and US patient summaries
2016	EU–US roadmap targets an IPS; CEN IPS work starts
2019	CEN EN 17269 published
2020	HL7 FHIR IPS 1.0
2021	ISO 27269; G7/GDHP IPS roadmap
2025	FHIR IPS 2.0 and ISO 27269:2025
2026	IPS Implementation Registry launched

4 Localised around the world

AU	**AU Patient Summary (AU PS)** Built on IPS 2.0.1 and AU Core; a valid AU PS is a valid IPS
CA	**PS-CA, Pan-Canadian Patient Summary** Infoway, Sept 2022; configurable by provinces, IPS-aligned
US	**US-PCS, US Patient Care Summary** In development; aligned with US Core and USCDI
EU	**HL7 Europe Patient Summary** STU1 ballot with Xt-EHR; basis for EHDS and MyHealth@EU
NZ	**NZ Patient Summary (HISO 10099:2022)** Conforms to ISO 27269 and the FHIR IPS
WHO	**SMART Verifiable IPS for Pilgrimage** Signed IPS read by QR code for Hajj pilgrims

5 What this means for countries and partners

Start with priority use cases

Use the IPS core where it adds most value, such as referrals and transfers of care. Add use-case data, e.g. the referral reason and request.

Agree the data first

Define a national or regional clinical data model (CDI) before choosing software. A FHIR profile represents the model; it does not replace it.

Plan terminology

Secure access to SNOMED CT and LOINC (national or regional licence) and a shared terminology service.

Identity, consent, trust

Patient matching, consent and a trust framework when summaries move between organisations or across borders.

Country signals: Canada released PS-CA in 2022; New Zealand published its patient summary standard in 2022; Brazil published its IPS initiative in 2024; Australia balloted its first IPS-based guide in August 2025.

Sources: HL7 FHIR IPS IG (hl7.org/fhir/uv/ips); international-patient-summary.net; ISO 27269; HL7 AU PS (hl7.org.au/fhir/ps); Canada Health Infoway; HL7 US-PCS CI build; HL7 Europe PS (hl7.eu/fhir/eps); Te Whatu Ora HISO 10099; WHO SMART IPS for Pilgrimage; CORDIS (Trillium Bridge); HHS–EC eHealth roadmap (2016).

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